

User Acceptance Testing Guide

Role: Principal Investigator (PI) — demo persona “Dr. Ananya Sharma”

Document date: 12 September 2026

Application under test: **<https://ayusphere-three.vercel.app>**

Environment: Production demo (seeded demo data — no real patient data)

YOUR TEST CREDENTIALS

URL

Email

Password

This guide walks you, step by step, through every feature available to the **Principal Investigator (PI)** role. No technical knowledge is required — follow the numbered steps, compare what you see with the screenshot, tick the checklist, and record Pass / Fail in the sign-off table at the end. All screenshots were captured from the live application.

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About this role

What the PI can do: full trial management (create trials, drive the lifecycle, register on CTRI, activate, close), site management, participant screening / consent / enrolment / withdrawal, CRF data entry *and approval* (the PI and the Administrator are the only roles that approve CRF entries and AI extractions), adverse-event capture, alert acknowledgement, AI Copilot, documents, and submission-ready exports.

What the PI cannot do: approve a trial through Ethics (IEC) review — that gate belongs to the Ethics Committee; run the pharmacovigilance review of adverse events — that belongs to the PV role; manage users or view the audit trail — those belong to the Administrator (and the Regulator, for audit).

What you will test

1. Logging in
2. Navigation overview & dashboard
3. Clinical Trials — portfolio list
4. Trial detail & lifecycle control
5. Participants — screening, consent, enrolment
6. Visit Schedule
7. Visit detail — the electronic CRF
8. Data Entry (eCRF) — Doctor Note Intelligence (AI)
9. Extractions — AI pipeline transparency
10. Adverse Events — capture
11. Alerts — acknowledge
12. AI Assistant (Copilot)
13. Messages (internal communication)
14. Documents
15. Reports & Exports (submission-ready)
16. Signing out

Before you start

General notes for testers:

- Use a modern desktop browser (Chrome or Edge recommended) at a comfortable window size.
- This is a shared demo environment — other testers may add data while you test; counts in your screenshots may differ slightly from the figures in this guide. That is normal and not a defect.

- Every action you take is recorded in the immutable audit trail — this is by design (ALCOA+ compliance).
- AI features (Doctor Note extraction and the AI Copilot) are powered by a configurable AI provider — currently Google Gemini (gemini-2.5-flash); an administrator can switch to Anthropic Claude under Settings. AI output can vary between runs; judge behaviour, not exact wording.
- If a page fails to load, refresh once before recording a failure (cold serverless starts can take a few seconds).

1 Logging in

What this is for. AyuSphere is protected by role-based access control (RBAC). Signing in with your role account determines exactly which pages and actions you can see — nothing more, nothing less.

STEPS

1. Open a browser and go to `https://ayusphere-three.vercel.app`. You are redirected to the login page.
2. In the **Email** field type `pi@aiaa.demo`.
3. In the **Password** field type `Demo@1234`.
4. Click the **Sign in** button.

Expected result. You are taken to the **Dashboard**. The top-right corner shows “Dr. Ananya Sharma — Principal Investigator”.

ACCEPTANCE CHECKLIST

- ☐ The login page loads with AyuSphere branding.
- ☐ A wrong password shows an error and does NOT log you in (try once, then use the correct one).
- ☐ The correct credentials open the dashboard.
- ☐ Your name and role are shown in the top-right corner.

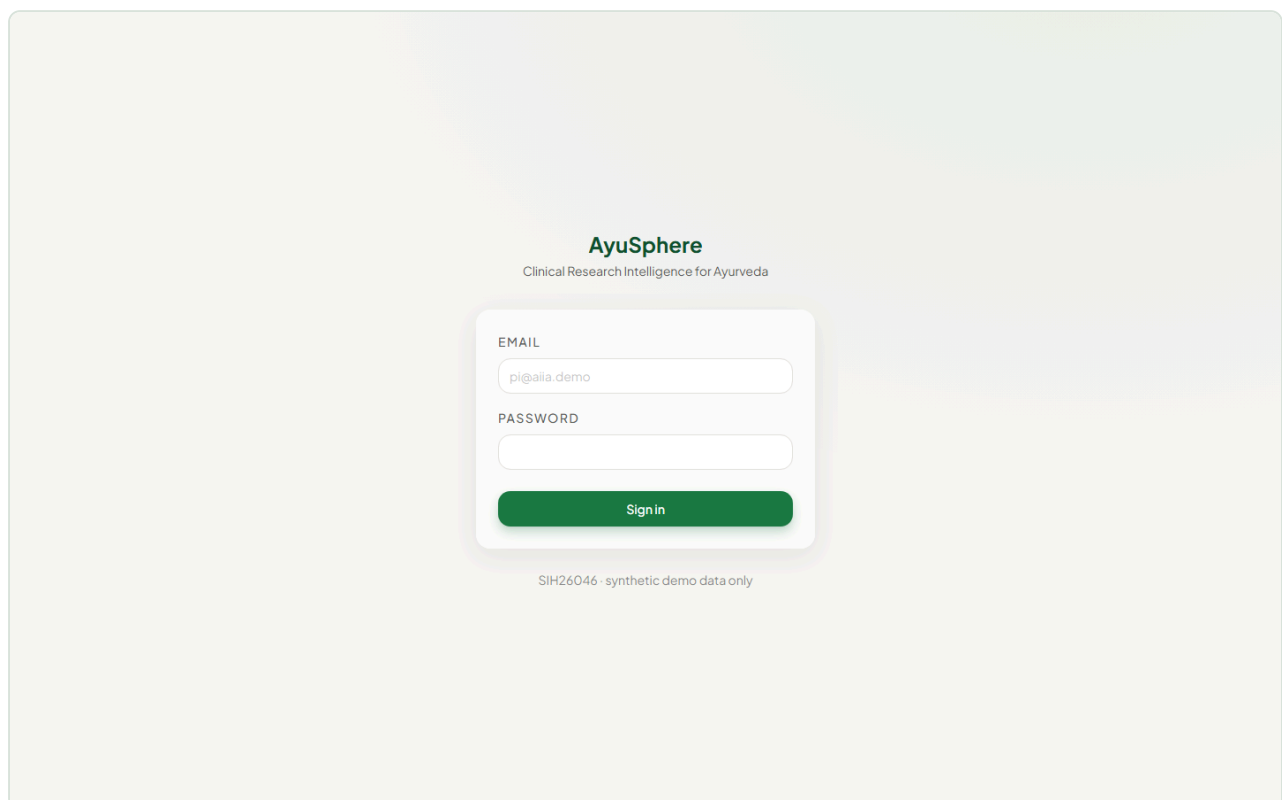


Fig. 1. The AyuSphere login page. Captured from the live system on 12 September 2026.

2 Navigation overview & dashboard

What this is for. The dashboard is your daily cockpit: portfolio KPIs, trial progress, participant distribution, site recruitment performance, upcoming/overdue visits, recent activity, AI insights, and a built-in AI Assistant panel.

STEPS

1. After logging in, look at the left sidebar. As PI you should see: **Dashboard, Messages, Clinical Trials, Participants, Site Management, Visit Schedule, Data Entry (eCRF), Extractions, Adverse Events, Alerts, AI Assistant, Documents, Reports & Exports.**
2. On the dashboard, review the KPI tiles across the top: Active Trials, Total Participants, Sites, Upcoming Visits, Adverse Events.
3. Scroll down: check the **Trial Progress** chart, **Participant Distribution** donut, and **Site Performance** recruitment rates.
4. Check the **Upcoming Visits** card — overdue visits are flagged in red (e.g. "17d overdue").
5. Check the **AI Insights** card (dropout risk, recruitment lag, protocol deviations, safety signal).
6. Note the **AI Assistant** panel on the right with a Chat tab and a Document-to-Data upload tab.

Expected result. All widgets render with data from the seeded demo trials. The sidebar shows exactly the 13 PI menu items — no Ethics Review, Monitoring, Audit Trail, or Settings entries.

ACCEPTANCE CHECKLIST

- ☐ The sidebar shows exactly the menu items listed above — no more, no fewer.
- ☐ The welcome header greets you by name.
- ☐ KPI tiles show numbers (not errors or blank tiles).
- ☐ The current date is displayed correctly.
- ☐ Overdue visits are visibly flagged.
- ☐ The AI Insights card is present.

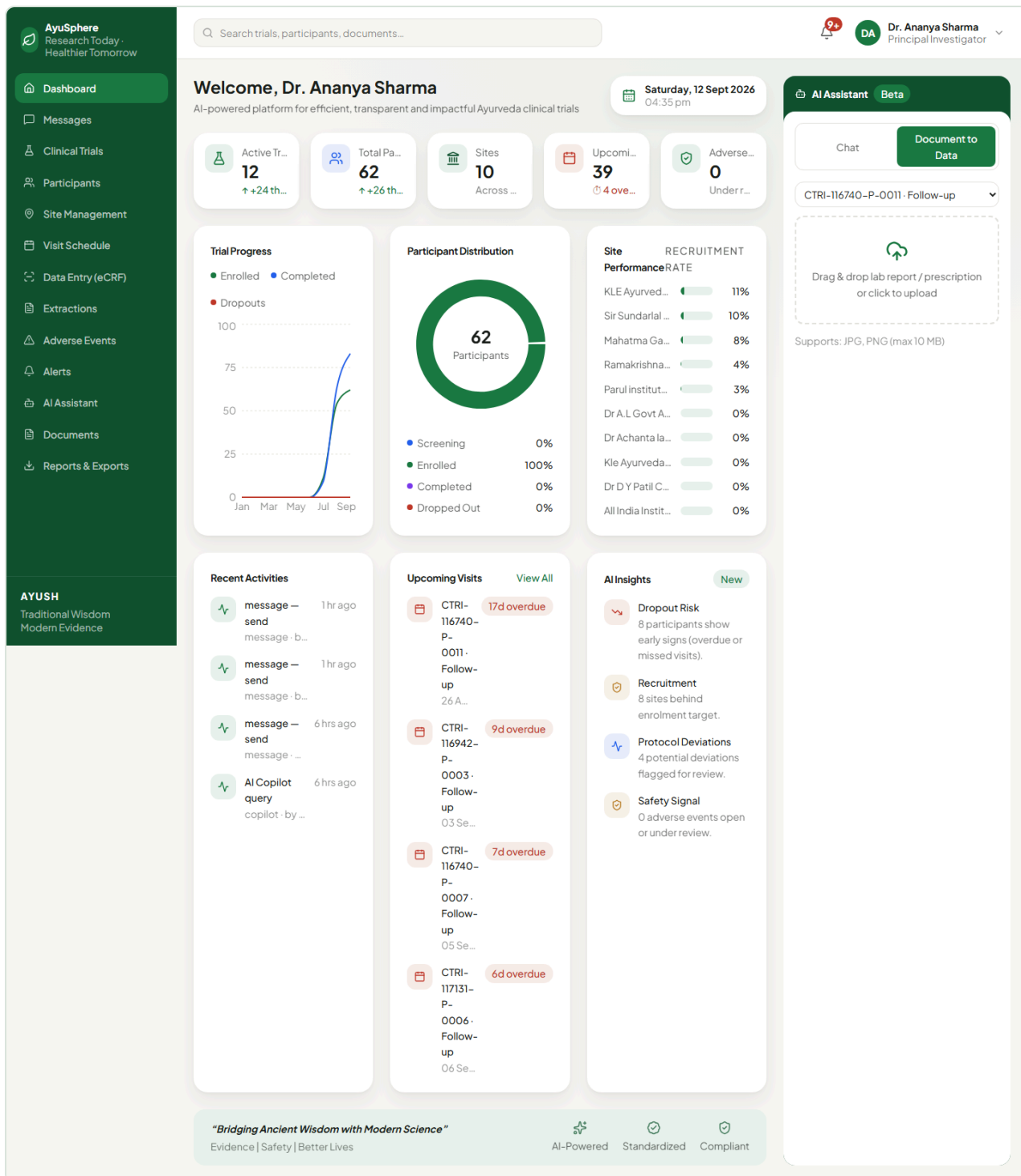


Fig. 2. PI dashboard. Captured from the live system on 12 September 2026.

3 Clinical Trials — portfolio list

What this is for. One list of every trial in your portfolio with its lifecycle status (draft → IEC review → IEC approved → CTIR registered → active → close-out), enrolment progress, phase and site count. This is also where a new trial is created.

STEPS

1. Click **Clinical Trials** in the sidebar.
2. Read the subtitle: "Full lifecycle: protocol → IEC → CTIR → activation → close-out".
3. Scan the trial cards: each shows the CTIR number, a status chip (e.g. *active*, *iec approved*, *ctir registered*), the registration date, title, intervention, phase, site count and an enrolment bar.
4. Locate the **New Trial** button (top right) — click it only if your test plan includes creating a trial; it opens a protocol form.

Expected result. The seeded portfolio (24+ Ayurveda trials, e.g. CTIR-117131 shatapushpa taila matrabasti, CTIR-116740 Panchakarma for neck pain) is listed with status chips and enrolment bars.

ACCEPTANCE CHECKLIST

- ☐ Every trial card shows CTIR ID, status chip, title, intervention and phase.
- ☐ Enrolment bars show a count and percentage.
- ☐ The New Trial button is visible to the PI.

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Clinical Trials

Full lifecycle: protocol → IEC → CTRI → activation → close-out.

New Trial

CTRI-117286

ctri registered

CTRI/2026/09/117286

Comparison of Grape Seed Oil and Aloe Vera Preparation with Clotrimazole for Reducing Oral Candida in Postmenopausal Women with Xerostomia

Grape seed oil with Aloe vera extract · interventional · Phase II · 1 site

ENROLMENT 0/110

0%

CTRI-117131

active

CTRI/2026/09/117131

Comparison of shatapushpa taila matrabaṣṭi with tila taila matrabaṣṭi in the management of kashtartava or primary dysmenorrhea

Shatapushpa taila; mātrābāṣṭi, मात्राबस्ति · interventional · Phase II/III · 1 site

ENROLMENT 14/138

10%

CTRI-116942

iec approved

CTRI/2026/09/116942

Pilot Clinical Evaluation of a Herbo-mineral Hair Removal Cream for Effective Hair Removal

Herbo mineral hair removal cream · interventional · 1 site

ENROLMENT 5/20

25%

CTRI-117332

ctri registered

CTRI/2026/09/117332

A clinical study to evaluate the safety and efficacy of tab Endo-HYLM, a herbomineral formulation in the management of female patients with endometrioma

Endo-HYLM · interventional · Phase I/II · 1 site

ENROLMENT 10/70

14%

CTRI-117085

active

CTRI/2026/09/117085

Role of Panchakarma in treating low back pain

pīṇḌasvedāḥ, पिण्डस्वेदः · interventional · Phase II/III · 1 site

ENROLMENT 0/45

0%

CTRI-116719

ctri registered

CTRI/2026/08/116719

Efficacy of Bala Taila Abhyanga in Occupational Musculoskeletal Disorders among Cobblers: A Pilot Study

Bala Taila; aBya-gga-karma अङ्ग-कर्म; Yogaraja Guggulu; Maharasnadi Kwatha; Ashwagandha Churna · interventional · 1 site

ENROLMENT 0/15

0%

CTRI-117339

iec approved

CTRI/2026/09/117339

A Study to Compare an Ayurvedic Medicine, Tamra Bhasma with Trikatu Kashaya for Improving Blood Cholesterol Levels

Tamra Bhasam · interventional · Phase IV · 1 site

ENROLMENT 0/80

0%

CTRI-116709

active

CTRI/2026/08/116709

Comparison Of Modified And Classical Takradhara In Management Of Psoriasis Skin Disorder

Vishaghna Mahakashaya; takradhArA, तक्रधरा · interventional · Phase III/IV · 1 site

ENROLMENT 0/24

0%

CTRI-116740

ctri registered

CTRI/2026/08/116740

Role of Panchakarma treatment in Neck pain

nasyam (viShopakramaḥ), नस्यम् (विशोपक्रमः) · interventional · Phase II · 1 site

ENROLMENT 11/40

28%

CTRI-116585

active

CTRI/2026/08/116585

Role of therapeutic Emesis and Purgative treatment on female Hormones and their activity.

vamana-karma, वमन-कर्म; virecana-karma, विरेचन-कर्म · interventional · Phase II · 1 site

ENROLMENT 11/40

28%

Fig. 3. Clinical Trials list — long page cropped to fit; scroll in the live app to see the rest. Captured from the live system on 12 September 2026.

4 Trial detail & lifecycle control

What this is for. Each trial has a detail page with its lifecycle action, KPIs (enrolment, dropout rate, visit compliance), per-site performance, milestone history and document shortcuts. The PI drives every lifecycle transition except the Ethics (IEC) decision.

STEPS

1. From **Clinical Trials**, click any trial card (the screenshot shows CTRI-117286, a CTRI-registered trial).
2. Check the header: CTRI ID, full title, intervention, phase, and current status chip.
3. In the **Lifecycle** card, note the single next-step action offered (e.g. **Activate trial** for a CTRI-registered trial, or **Submit for IEC review** for a draft). Click it only if your test plan includes advancing this trial.
4. Review the three KPI cards: Enrolment, Dropout Rate, Visit Compliance.
5. Review **Site performance** — sites behind their enrolment target carry a red *behind* chip.
6. Review **Milestones** (IEC approval, CTRI registration dates) and the **Documents** card.
7. Click ← **Clinical Trials** to return to the list.

Expected result. The detail page shows the correct trial, exactly one lifecycle action appropriate to its current status, live KPIs, site performance and milestone dates. A trial in IEC review offers the PI no approval button — only the Ethics Committee can approve it.

ACCEPTANCE CHECKLIST

- ☐ Opening a trial card navigates to its detail page.
- ☐ The lifecycle card offers only the valid next transition.
- ☐ Enrolment / dropout / visit-compliance KPIs are populated.
- ☐ Milestones show IEC approval and CTRI registration dates where reached.
- ☐ The back link returns to the list.

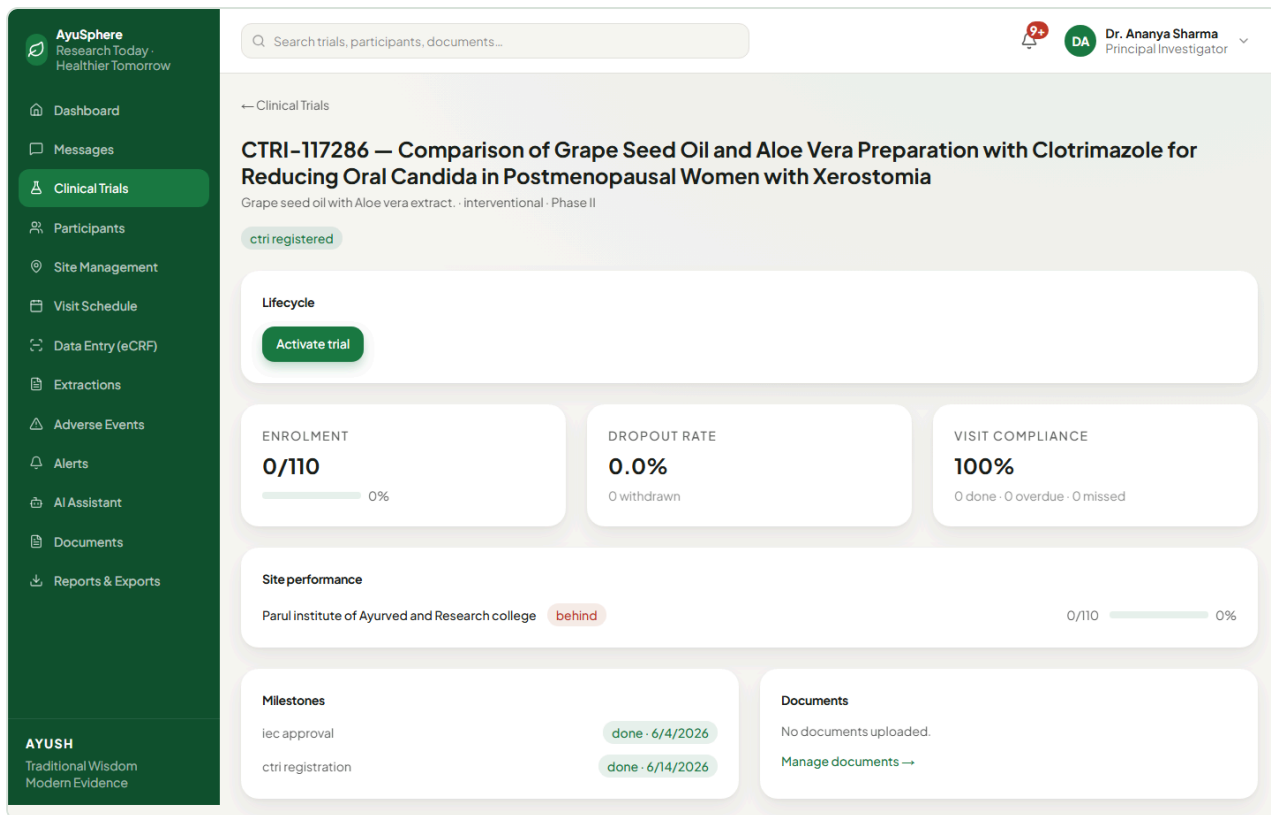


Fig. 4. Trial detail page with lifecycle action and KPIs. Captured from the live system on 12 September 2026.

5 Participants — screening, consent, enrolment

What this is for. De-identified participant register (subject codes only — no personal data). Participants move through screening → consent → enrolment → visits, and can be withdrawn with a reason. The PI can add participants to an active trial with an auto-generated code.

STEPS

1. Click **Participants** in the sidebar.
2. In **Add participant**, note the Active Trial · Site selector and the **Add (code auto-generated)** button — clicking it screens a new de-identified participant into the selected trial.
3. Use **Filter by trial** to narrow the table to one trial and click **Apply**.
4. Read one row: subject code (e.g. CTRI-116585-P-0001), trial & site, screening chip (*passed*), consent chip (*given*), status (*enrolled* — intervention or control arm).
5. Note the **Withdraw** action with its reason field — do NOT withdraw anyone unless your test plan says so.

Expected result. Only de-identified subject codes are shown (no names or personal data). Each row carries screening, consent and enrolment status with an arm label; a withdraw action with mandatory reason is available.

ACCEPTANCE CHECKLIST

- ☐ No personally identifying data appears anywhere in the register.
- ☐ The trial filter narrows the list correctly.
- ☐ Each participant shows screening, consent and status chips.
- ☐ The add-participant control auto-generates the subject code.

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Participants

De-identified subject codes only — screening → consent → enrolment → visits.

Add participant

ACTIVE TRIAL · SITE

CTRI-117425 · Mahatma Gandhi Ayurved College Hospital and Research Centre, Wardha

Add (code auto-generated)

FILTER BY TRIAL

All trials (6)

Apply

62 participants

SUBJECT	TRIAL · SITE	SCREENING	CONSENT	STATUS	ACTIONS
CTRI-116585-P-0001	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0002	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled control	withdrawal reason Withdraw
CTRI-116585-P-0003	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0004	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0005	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0006	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0007	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0008	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0009	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled control	withdrawal reason Withdraw
CTRI-116585-P-0010	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116585-P-0011	CTRI-116585 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116740-P-0001	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled control	withdrawal reason Withdraw
CTRI-116740-P-0002	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled control	withdrawal reason Withdraw
CTRI-116740-P-0003	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116740-P-0004	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116740-P-0005	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw
CTRI-116740-P-0006	CTRI-116740 · KLE Ayurveda Hospital and MRC	passed	given	enrolled intervention	withdrawal reason Withdraw

Fig. 5. Participants register — long page cropped to fit; scroll in the live app to see the rest. Captured from the live system on 12 September 2026.

6 Visit Schedule

What this is for. Protocol visit windows are computed automatically from each participant's enrolment date. The schedule groups visits into Overdue, Due and Upcoming so the site team always knows what to capture next.

STEPS

1. Click **Visit Schedule** in the sidebar.
2. Check the three groups: **OVERDUE**, **DUE** and **UPCOMING**, each row showing subject code, visit name (Baseline / Follow-up), a status chip and the protocol window plus scheduled date.
3. Click any visit row (e.g. an overdue Follow-up) to open its detail page — the next section tests it.

Expected result. Visits are correctly grouped by their protocol window relative to today's date; each row shows window dates and the scheduled date; clicking a row opens the visit's CRF page.

ACCEPTANCE CHECKLIST

- ☐ The three groups (Overdue, Due, Upcoming) are present with counts.
- ☐ Every row shows a protocol window and scheduled date.
- ☐ Clicking a visit opens its detail page.



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Visit Schedule
 Protocol windows computed from enrolment — open a visit to capture its CRF.

OVERDUE (4)

CTRI-116740-P-0011

Follow-up

overdue

window 8/19/2026 – 9/2/2026 · scheduled 8/26/2026

CTRI-116942-P-0003

Follow-up

overdue

window 8/27/2026 – 9/10/2026 · scheduled 9/3/2026

CTRI-116740-P-0007

Follow-up

overdue

window 8/29/2026 – 9/12/2026 · scheduled 9/5/2026

CTRI-116585-P-0003

Baseline

overdue

window 9/6/2026 – 9/12/2026 · scheduled 9/9/2026

DUE (22)

CTRI-117131-P-0006

Follow-up

due

window 8/30/2026 – 9/13/2026 · scheduled 9/6/2026

CTRI-116740-P-0008

Follow-up

due

window 9/1/2026 – 9/15/2026 · scheduled 9/8/2026

CTRI-117131-P-0002

Follow-up

due

window 9/1/2026 – 9/15/2026 · scheduled 9/8/2026

CTRI-117332-P-0010

Follow-up

due

window 9/2/2026 – 9/16/2026 · scheduled 9/9/2026

CTRI-116740-P-0009

Follow-up

due

window 9/2/2026 – 9/16/2026 · scheduled 9/9/2026

CTRI-117131-P-0001

Follow-up

due

window 9/2/2026 – 9/16/2026 · scheduled 9/9/2026

CTRI-116585-P-0004

Follow-up

due

window 9/2/2026 – 9/16/2026 · scheduled 9/9/2026

CTRI-117253-P-0006

Follow-up

due

window 9/2/2026 – 9/16/2026 · scheduled 9/9/2026

CTRI-117332-P-0009

Follow-up

due

window 9/3/2026 – 9/17/2026 · scheduled 9/10/2026

CTRI-116585-P-0001

Follow-up

due

window 9/4/2026 – 9/18/2026 · scheduled 9/11/2026

CTRI-117131-P-0014

Follow-up

due

window 9/5/2026 – 9/19/2026 · scheduled 9/12/2026

CTRI-116585-P-0002

Follow-up

due

window 9/5/2026 – 9/19/2026 · scheduled 9/12/2026

CTRI-116740-P-0003

Follow-up

due

window 9/5/2026 – 9/19/2026 · scheduled 9/12/2026

CTRI-117131-P-0003

Follow-up

due

window 9/7/2026 – 9/21/2026 · scheduled 9/14/2026

CTRI-117253-P-0010

Follow-up

due

window 9/8/2026 – 9/22/2026 · scheduled 9/15/2026

CTRI-116740-P-0004

Follow-up

due

window 9/8/2026 – 9/22/2026 · scheduled 9/15/2026

CTRI-116740-P-0001

Follow-up

due

window 9/8/2026 – 9/22/2026 · scheduled 9/15/2026

Fig. 6. Visit Schedule — long page cropped to fit; scroll in the live app to see the rest. Captured from the live system on 12 September 2026.

7 Visit detail — the electronic CRF

What this is for. Each visit has an electronic Case Report Form (e-CRF) with CDISC-mapped fields (systolic/diastolic BP, pulse, intervention dose, clinical notes). Data can be saved as a draft or submitted for approval; as PI you also approve submitted entries. Completing the visit stamps it and clears its overdue alerts.

STEPS

1. From **Visit Schedule**, open a visit (the screenshot shows CTRI-116740-P-0011 — Follow-up, flagged *overdue*).
2. Check the header: subject code, visit name, protocol window, status chip.
3. In the **e-CRF** card, type plausible values: Systolic BP (70–250 mmHg), Diastolic BP (40–150), Pulse (30–200 bpm), Intervention dose (0–2000 mg), and an optional clinical note. Note the CDISC variable hints under each field (VSORRES_SYSBP, EXDOSE, ...).
4. Try an implausible value (e.g. pulse 999) — the form must refuse it with a validation message; correct it.
5. Click **Save draft** to store it, or **Save & submit for approval** to send it into the approval queue. As PI, a submitted entry then shows an approve action in the Entries list below.
6. Note the **or scan a doctor note** → shortcut (tested in the next section) and the **Mark visit completed** action at the bottom.

Expected result. The CRF accepts values inside the plausible ranges only. Saved entries appear in the Entries list with their status (draft / submitted / approved). Marking the visit completed stamps it and clears its overdue alert.

ACCEPTANCE CHECKLIST

- ☐ All 5 CRF fields render with CDISC hints and plausible-range captions.
- ☐ Out-of-range values are rejected with a clear message.
- ☐ Save draft and Save & submit for approval both work.
- ☐ The PI can approve a submitted entry (entries list).
- ☐ Mark visit completed changes the visit status.

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← Visit Schedule

CTRI-116740-P-0011 — Follow-up

Window 8/19/2026 – 9/2/2026

overdue

e-CRF — CTRI-116740 — Follow-up CRF

or scan a doctor note →

SYSTOLIC BP (MMHG) *

plausible 70–250 mmHg · VSORRES_SYSBP

DIASTOLIC BP (MMHG) *

plausible 40–150 mmHg · VSORRES_DIABP

PULSE (BPM) *

plausible 30–200 bpm · VSORRES_PULSE

INTERVENTION DOSE (MG) *

plausible 0–2000 mg · EXDOSE

CLINICAL NOTES

Save & submit for approval

Save draft

Entries (0)

No CRF entries yet.

Done capturing? Completing the visit stamps it and clears overdue alerts.

Mark visit completed

Fig. 7. Visit detail with the e-CRF form. Captured from the live system on 12 September 2026.

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8 Data Entry (eCRF) — Doctor Note Intelligence (AI)

What this is for. The flagship AI feature: photograph a handwritten trial note and the platform (Google Gemini vision, currently gemini-2.5-flash) drafts the visit CRF from it — with per-field confidence scores and full provenance. Nothing becomes an official record until a human approves it; low-quality photos are rejected with a “recapture needed” message.

STEPS

1. Click **Data Entry (eCRF)** in the sidebar — the “Doctor Note Intelligence” page opens.
2. In **1 · Capture**, pick a participant visit from the dropdown.
3. Under **Note photo**, choose a photo of a handwritten note (JPG/PNG). A sample note photographing BP, pulse and dose works best.
4. Click **Extract to CRF draft** and wait while it reads the note (a “Reading note...” state shows).
5. In **2 · Review draft**, check each extracted field against the photo: value, confidence percentage, and the source text it was read from. Fields below the confidence threshold are flagged “confirm” and must be verified by you.
6. Correct any field if needed, then click **Approve as record** (a PI/Admin-only action) — or discard the draft.

Expected result. A legible note extracts into a CRF draft with high confidences; approving creates an official CRF record with full provenance (“Approved — official CRF record created with full provenance”). A blurred or non-clinical image is rejected with a recapture message instead of guessing.

ACCEPTANCE CHECKLIST

- ☐ The visit selector and photo upload are present.
- ☐ Extraction produces a draft with per-field confidence badges.
- ☐ Each field shows the source text it was read from (provenance).
- ☐ Approve as record is available to the PI and creates an official entry.
- ☐ A poor-quality image is rejected, not silently accepted.

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Doctor Note Intelligence

Photograph a handwritten trial note — the platform drafts the visit CRF, validates it, and nothing becomes a record until you approve it.

1 · Capture

PARTICIPANT VISIT

CTRI-116740-P-0011 · Follow-up · 8/26/2026

NOTE PHOTO

Choose File No file chosen

Extract to CRF draft

Fig. 8. Doctor Note Intelligence — capture step. Captured from the live system on 12 September 2026.

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9 Extractions — AI pipeline transparency

What this is for. Every scanned doctor note ever processed, in one gallery: the original image, model used, per-extraction average confidence, timestamps and outcome (approved / review / rejected). This is the transparency and audit view of the AI pipeline.

STEPS

1. Click **Extractions** in the sidebar.
2. Review the cards: each shows the note thumbnail, an outcome chip (*approved* / *review* / *rejected*), the subject code and visit, field count, average confidence and the model name (gemini-2.5-flash).
3. Find a *rejected* card — it shows “no fields extracted” for an unreadable photo.
4. Click **Open visit & entries** → on any card to jump to the underlying visit.

Expected result. The gallery lists all extractions with images, statuses, confidences and model attribution; approved ones carry a review date; rejected ones show why nothing was extracted.

ACCEPTANCE CHECKLIST

- ☐ Each extraction card shows the original note image.
- ☐ Status chips distinguish approved / review / rejected.
- ☐ Average confidence and model name are displayed.
- ☐ The link to the underlying visit works.

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Dr. Ananya Sharma
Principal Investigator

Extractions

Every scanned document: original image, model output, confidences, and outcome — the full Doctor Note pipeline at a glance.

Document ID	Status	Pipeline Stage	Fields	Confidence	Model	Flash	Date	Time	Action
CTRI-117131-P-0012	approved	Follow-up	5 fields	avg confidence 99%	gemiini-2.5	-flash	9/12/2026	9:49:45 AM	reviewed 9/12/2026
Open visit & entries →									
CTRI-117131-P-0012	approved	Follow-up	5 fields	avg confidence 98%	gemiini-2.5	-flash	9/12/2026	9:42:32 AM	reviewed 9/12/2026
Open visit & entries →									
CTRI-117131-P-0012	approved	Follow-up	5 fields	avg confidence 99%	gemiini-2.5	-flash	9/12/2026	9:30:15 AM	reviewed 9/12/2026
Open visit & entries →									
CTRI-117131-P-0012	approved	Follow-up	5 fields	avg confidence 98%	gemiini-2.5	-flash	9/12/2026	9:23:20 AM	reviewed 9/12/2026
Open visit & entries →									
CTRI-117131-P-0006	review	Follow-up	5 fields	avg confidence 100%	gemiini-2.5	-flash	9/12/2026	9:11:21 AM	
Open visit & entries →									
CTRI-117131-P-0003	review	Follow-up	5 fields	avg confidence 97%	gemiini-2.5	-flash	9/12/2026	8:52:11 AM	
Open visit & entries →									
CTRI-117131-P-0002	rejected	Follow-up	no fields extracted	avg confidence -	gemiini-2.5	-flash	9/12/2026	8:49:36 AM	
Open visit & entries →									
CTRI-117131-P-0001	review	Follow-up	5 fields	avg confidence 100%	gemiini-2.5	-flash	9/12/2026	8:45:46 AM	
Open visit & entries →									

Fig. 9. Extractions gallery. Captured from the live system on 12 September 2026.

10 Adverse Events — capture

What this is for. Capture safety events (AE/SAE) against a participant. Captured events start an escalation clock from the moment of capture and are queued for the Pharmacovigilance role to review, report and close — the PI captures; PV reviews.

STEPS

1. Click **Adverse Events** in the sidebar.
2. Read the page: open AE/SAEs are sorted by reporting deadline with a countdown from capture time.
3. If the list is empty, the empty state reads “No open adverse events. Captured events appear here with their reporting countdown.”
4. Use the capture control to record a test AE only if your test plan includes safety-flow testing (it creates a real record and starts the PV escalation clock).

Expected result. The AE queue is visible with deadline sorting. A newly captured event appears with severity, description and its reporting countdown, and becomes visible to the PV role for review.

ACCEPTANCE CHECKLIST

- ☐ The Adverse Events page opens with the deadline-sorted queue (or a clear empty state).
- ☐ The capture control is available to the PI.
- ☐ A captured event shows its reporting countdown.

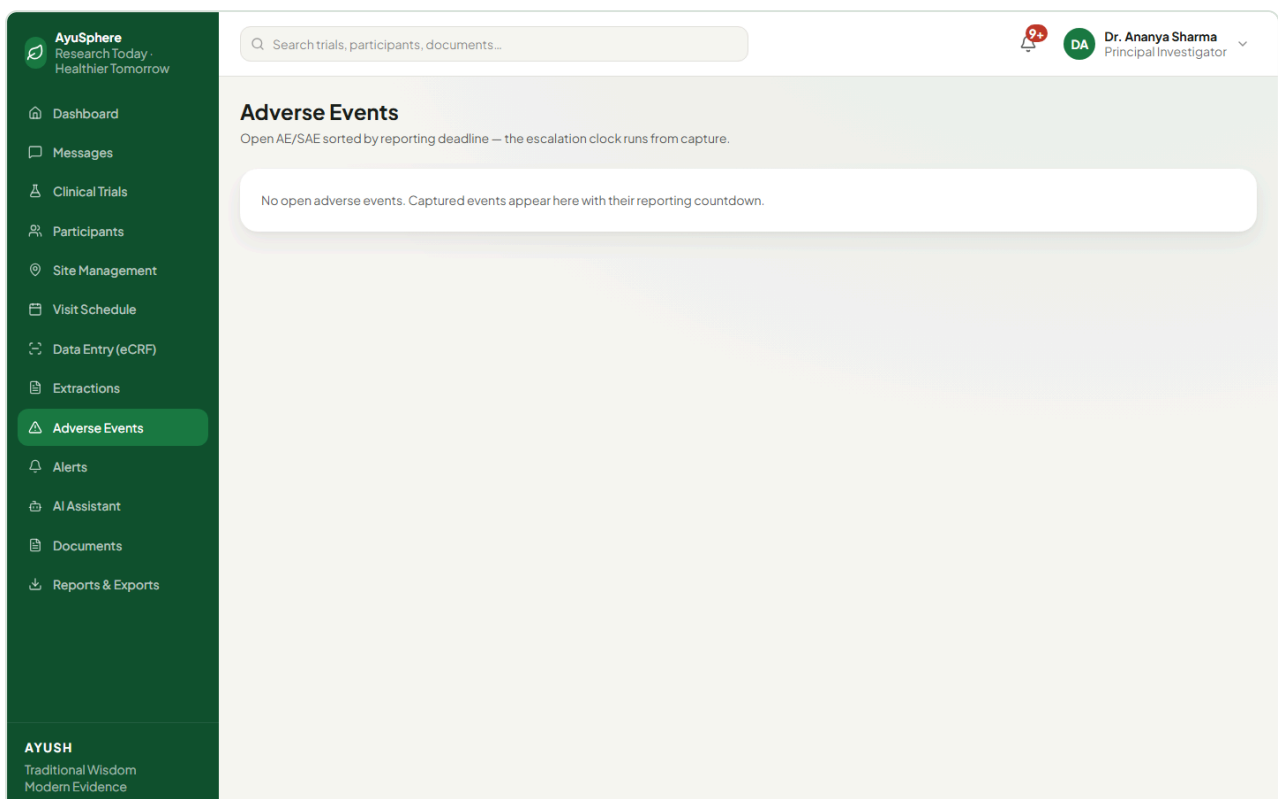


Fig. 10. Adverse Events queue. Captured from the live system on 12 September 2026.

11 Alerts — acknowledge

What this is for. The platform raises alerts automatically across your scope: enrolment lag, overdue visits, protocol deviations, data-quality findings (e.g. a visit completed without an approved CRF entry) and safety deadlines. Acknowledging an alert records who saw it and when.

STEPS

1. Click **Alerts** in the sidebar.
2. Review severities: *danger* (e.g. a missed visit — protocol deviation) versus *warning* (e.g. data-quality findings, enrolment behind target).
3. Pick one alert, read its description and timestamp, and click **Acknowledge**.

Expected result. Alerts are listed with severity chips, category (protocol_deviation, data_quality, enrolment_lag, visit_overdue) and timestamps. Acknowledging removes it from the open list and records the acknowledgement in the audit trail.

ACCEPTANCE CHECKLIST

- ☐ Alerts show severity, category and timestamp.
- ☐ Both danger and warning severities are represented.
- ☐ Acknowledge removes the alert from the open list.

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Dr. Ananya Sharma

Principal Investigator

Search trials, participants, documents...

Alerts

Open alerts across your scope — enrolment lag, overdue visits, safety deadlines, milestone dues, data quality.

danger

Visit 'Baseline' was missed — protocol deviation

protocol_deviation · 9/12/2026, 8:14:42 AM

Acknowledge

danger

Visit 'Baseline' was missed — protocol deviation

protocol_deviation · 9/12/2026, 8:14:41 AM

Acknowledge

danger

Visit 'Baseline' was missed — protocol deviation

protocol_deviation · 9/12/2026, 8:14:40 AM

Acknowledge

danger

Visit 'Follow-up' was missed — protocol deviation

protocol_deviation · 9/12/2026, 8:14:39 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Follow-up' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Follow-up' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

warning

Visit 'Baseline' completed without an approved CRF entry

data_quality · 9/12/2026, 9:59:09 AM

Acknowledge

Fig. 11. Alerts — long page cropped to fit; scroll in the live app to see the rest. Captured from the live system on 12 September 2026.

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12 AI Assistant (Copilot)

What this is for. A grounded AI copilot over your authorized trial data (Gemini-powered, provider-switchable by the admin). Every answer cites the records it used; when nothing supports an answer it says so instead of inventing one. It only sees data your role is allowed to see.

STEPS

1. Click **AI Assistant** in the sidebar.
2. Try a suggested prompt chip, e.g. **Which sites are behind target?**, or type your own question about recruitment, safety or the portfolio and click **Ask**.
3. Read the answer and verify it cites the underlying records it used.
4. Ask something the data cannot answer (e.g. "What is participant 0001's home address?") — it must refuse or state that nothing supports an answer, never fabricate.

Expected result. Answers arrive grounded in real trial data with citations. Unanswerable questions get an honest "no supporting data" response. De-identified design means no personal data can be revealed.

ACCEPTANCE CHECKLIST

- ☐ Suggested prompts are present and clickable.
- ☐ An answer is produced and cites its sources.
- ☐ An unanswerable question is refused honestly.

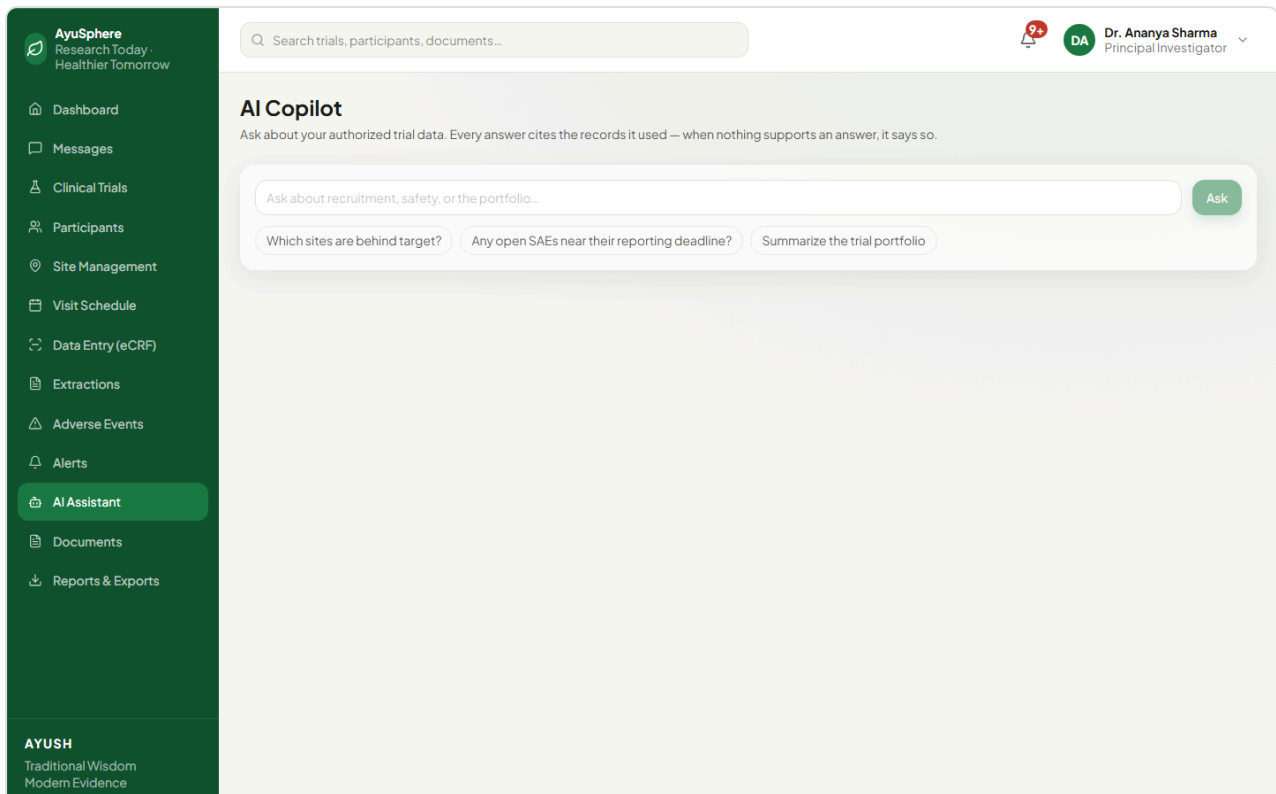


Fig. 12. AI Copilot. Captured from the live system on 12 September 2026.

13 Messages (internal communication)

What this is for. Direct one-to-one messaging with every other user on the platform — coordinators, investigators, monitors, ethics, pharmacovigilance, admin and the regulator — so trial communication stays inside the audited system.

STEPS

1. Click **Messages** in the left sidebar.
2. The **People** list on the left shows every other platform user with their role badge.
3. Click a person (for example the Study Coordinator) to open the conversation panel.
4. Type a short test message (e.g. “UAT test — please ignore”) and send it.
5. Confirm your message appears in the thread with a timestamp.

Expected result. The people list shows the other demo users with role badges. A sent message appears immediately in the conversation and is delivered to the recipient (message events are also recorded in the audit trail).

ACCEPTANCE CHECKLIST

- ☐ Messages page opens from the sidebar.
- ☐ All other demo users are listed with correct role badges.
- ☐ Selecting a person opens a chat panel.
- ☐ A test message can be sent and appears with a timestamp.

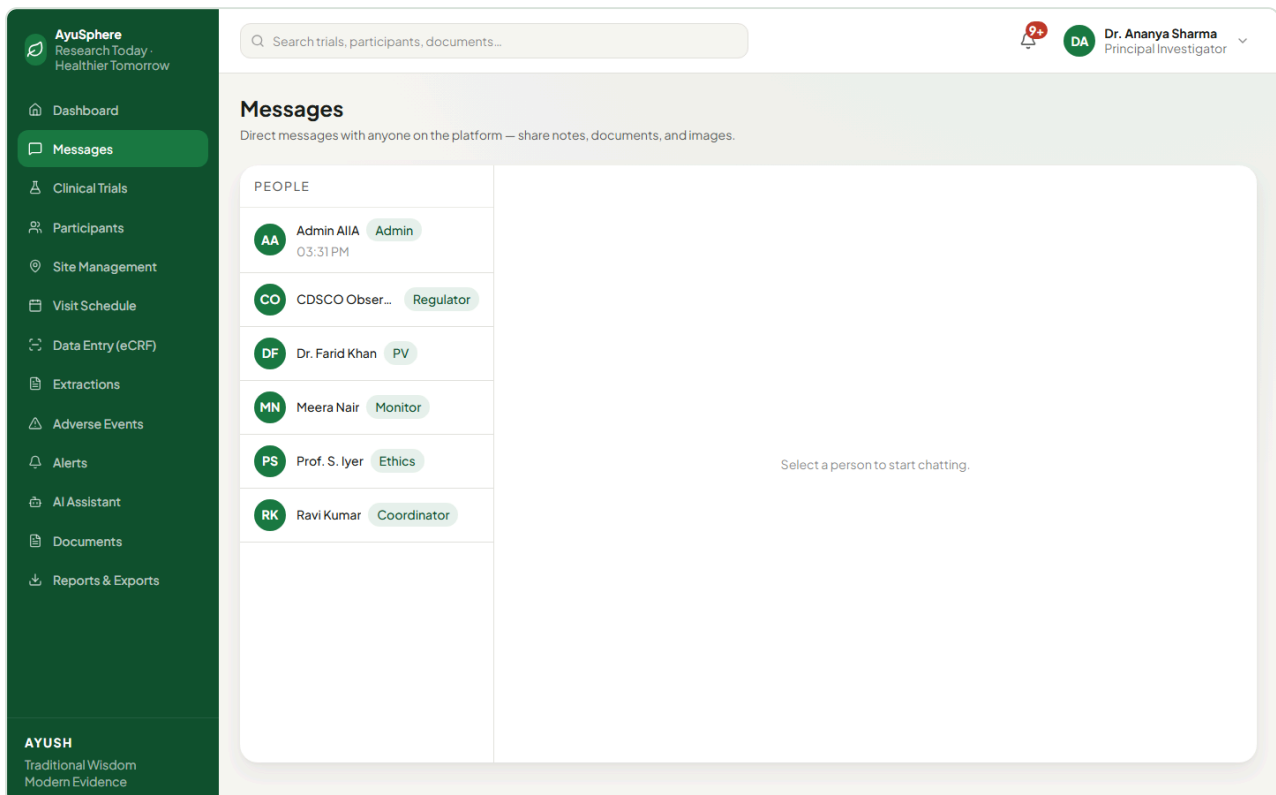


Fig. 13. Messages — people list and conversation panel. Captured from the live system on 12 September 2026.

14 Documents

What this is for. Central library for trial documents — protocols, ethics approvals, consent forms — attached to a trial with a kind and title. Version history is preserved; uploads are audited.

STEPS

1. Click **Documents** in the sidebar.
2. In the **Upload** card, select a trial, a kind (e.g. Protocol), type a title, choose a small PDF/image file (max 2 MB) and click **Upload**.
3. Confirm the document appears in the **Library** list below with its trial, kind, title and version.

Expected result. The upload succeeds and the file is listed in the library attached to the chosen trial. Re-uploading the same kind/title creates a new version rather than overwriting silently.

ACCEPTANCE CHECKLIST

- ☐ Upload form offers trial, kind, title and file inputs.
- ☐ A valid file uploads successfully.
- ☐ The library lists the document with its metadata.

The screenshot shows the AyuSphere web application interface. On the left is a dark green sidebar with the AyuSphere logo and tagline 'Research Today · Healthier Tomorrow'. Below the logo is a list of navigation items: Dashboard, Messages, Clinical Trials, Participants, Site Management, Visit Schedule, Data Entry (eCRF), Extractions, Adverse Events, Alerts, AI Assistant, Documents (highlighted in green), and Reports & Exports. At the bottom of the sidebar is the AYUSH logo with the tagline 'Traditional Wisdom · Modern Evidence'. The main content area has a light green header with a search bar containing the text 'Search trials, participants, documents...'. To the right of the search bar is a user profile for 'Dr. Ananya Sharma' with the role 'Principal Investigator'. Below the header is the 'Documents' section, which includes a sub-header 'Protocols, ethics approvals, consent forms — version history preserved.' and an 'Upload' card. The 'Upload' card has four input fields: 'TRIAL' (a dropdown menu with 'CTRI-116584' selected), 'KIND' (a dropdown menu with 'Protocol' selected), 'TITLE' (a text input field with 'Protocol v2'), and 'FILE (MAX 2 MB)' (a button that says 'Choose File No file chosen'). To the right of these fields is a green 'Upload' button. Below the 'Upload' card is a 'Library (0)' section with the text 'No documents uploaded yet.'

Fig. 14. Documents library. Captured from the live system on 12 September 2026.

15 Reports & Exports (submission-ready)

What this is for. Per-trial regulatory export bundles in standard formats: a FHIR R4 bundle (ResearchStudy, ResearchSubject with de-identified patients, AdverseEvent), SDTM DM domain (CSV), SDTM AE domain (CSV) and a Define-XML stub. Every export run is recorded in the audit trail.

STEPS

1. Click **Reports & Exports** in the sidebar.
2. Find any trial block and note its four export buttons: **FHIR R4 Bundle**, **SDTM DM (CSV)**, **SDTM AE (CSV)**, **Define-XML stub**.
3. Click **SDTM DM (CSV)** for one trial — a CSV file downloads.
4. Open the downloaded file and confirm it contains de-identified subject rows for that trial.

Expected result. Each trial offers all four export formats; a clicked export downloads a well-formed file containing only de-identified data, and the run is logged in the audit trail.

ACCEPTANCE CHECKLIST

- ☐ Every trial block shows FHIR, SDTM DM, SDTM AE and Define-XML options.
- ☐ An export downloads successfully.
- ☐ The exported file contains de-identified data only.

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Principal Investigator

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Submission-Ready Exports

CDISC/FHIR focused subset — every export is recorded in the audit trail. Full SDTM/ADaM coverage is a roadmap item, stated plainly.

CTRI-117131 — Comparison of shatapushpa taila matrabasti with tila taila matrabasti in the management of kashtartava or primary dysmenorrhea

active

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

CTRI-117332 — A clinical study to evaluate the safety and efficacy of tab Endo-HYLM, a herbomineral formulation in the management of female patients with endometrioma

ctri_registered

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

CTRI-117253 — A Study Comparing Two Ayurvedic Ghee Preparations Prapoundarikadi Ghrita and Jatyadi Ghrita for Healing Dusta Vrana in Adults

active

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

CTRI-116942 — Pilot Clinical Evaluation of a Herbo-mineral Hair Removal Cream for Effective Hair Removal

iec_approved

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

CTRI-116585 — Role of therapeutic Emesis and Purgative treatment on female Hormones and their activity.

active

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

CTRI-116740 — Role of Panchakarma treatment in Neck pain

ctri_registered

FHIR R4 Bundle

ResearchStudy · ResearchSubject · Patient (de-identified) · AdverseEvent

SDTM DM (CSV)

Demographics domain

SDTM AE (CSV)

Adverse events domain

Define-XML stub

Metadata for the exported domains

Fig. 15. Submission-Ready Exports — long page cropped to fit; scroll in the live app to see the rest. Captured from the live system on 12 September 2026.

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16 Signing out

What this is for. Ends your session so nobody else can act under your account — important on shared computers at trial sites.

STEPS

1. Click your **name / avatar** in the top-right corner of any page.
2. Choose **Sign out** from the menu.

Expected result. You are returned to the login page. Pressing the browser Back button does not re-open protected pages with your data.

ACCEPTANCE CHECKLIST

- ☐ The sign-out option is available from the top-right user menu.
- ☐ After signing out you land on the login page.
- ☐ Navigating directly to a protected page (e.g. /dashboard) redirects you to log in again.

UAT sign-off

Record the outcome of each test case. A test passes when every item of its acceptance checklist is satisfied.

ID	Step / feature	Expected	Pass / Fail	Remarks
TC-01	Logging in	Login succeeds; dashboard opens with correct name and role		
TC-02	Navigation overview & dashboard	Dashboard renders all widgets; sidebar shows the 13 PI items only		
TC-03	Clinical Trials — portfolio list	Trial list shows all trials with status chips and enrolment bars		
TC-04	Trial detail & lifecycle control	Trial detail shows KPIs, milestones, and one status-appropriate lifecycle action		
TC-05	Participants — screening, consent, enrolment	De-identified participant register with screening/consent/enrolment chips and filter		
TC-06	Visit Schedule	Visits grouped Overdue / Due / Upcoming with protocol windows; rows open detail		
TC-07	Visit detail — the electronic CRF	CRF validates ranges, saves draft/submit, PI can approve; completing visit clears alerts		
TC-08	Data Entry (eCRF) — Doctor Note Intelligence (AI)	AI extracts note to CRF draft with confidences; PI approval creates official record		
TC-09	Extractions — AI pipeline transparency	Extraction gallery shows image, status, confidence and model for every scan		
TC-10	Adverse Events — capture	AE queue lists open events by deadline; capture starts the escalation clock		
TC-11	Alerts — acknowledge	Alerts list with severities; acknowledging one records and clears it		
TC-12	AI Assistant (Copilot)	Copilot answers with citations; refuses unanswerable questions		
TC-13	Messages (internal communication)	People list shows all users; a test message sends and appears in the thread		
TC-14	Documents	Document uploads to a trial and appears in the library		

ID	Step / feature	Expected	Pass / Fail	Remarks
TC-15	Reports & Exports (submission-ready)	All 4 export formats download well-formed, de-identified files		
TC-16	Signing out	Session ends; protected pages no longer accessible		

Tester name & signature

Date

Reviewed by (QA lead)